

Chain access was blocked, so the options section below is computed from our own table with the decision boundary stated explicitly rather than quoted. Here is the deck.

PITCH — ATEC — VERDICT

ADJUDICATED — **Minimum starter, 0.5% of book, resting buy limit at or below \$9.00 (good-till-cancelled); no chase above \$9.00; second half of the position is gated on the Q3 print on 2026-10-29.** Red's rejection (7/10) was partially overturned by blue (7/10), and on audit no kill survived as fatal — what survived shrinks the position rather than blocking it.

What we believe

Alphatec sells spine implants and the instruments, imaging and nerve-monitoring that go with them. Its differentiating pitch is a way of doing lateral spine fusion with the patient face-down instead of on their side, which lets the surgeon do the front and back of the operation without flipping the patient.

Three things we believe:

1. The business is genuinely growing and the growth is real revenue, not accounting. Q2 2026 revenue was \$213.5M, up 15%, with surgical revenue of \$196M up 17% and case volume up 20% (8-K Exhibit 99.1, 2026-08-04). Those are audited-adjacent, filed numbers. Surgeons are switching to this company.

2. The profit improvement is not a stock-compensation trick, and the sharpest argument against this name got that backwards. Our red bench's headline kill was that the margin expansion is just add-back of stock comp. We checked it against the company's own reconciliation. Stock comp went from \$15.6M to \$18.1M — up 16%, against revenue up 15%. It is flat as a share of sales. So earnings *before* the add-back went from \$7.8M to \$17.8M, more than doubling, and the margin improvement excluding stock comp (+409 basis points) is within a rounding error of the headline improvement (+420bp). The stock comp did not cause the inflection. It is large in level, and that is a separate and legitimate complaint, but the causal claim was inverted.

3. It is the cheapest thing in its own peer group on the only multiple that works here. At \$9.425 the equity is worth about \$1.46B; add roughly \$457M of net debt and enterprise value is about \$1.91B — 2.17x this year's guided revenue, 2.35x trailing. Globus Medical trades near 3.5x growing under 7%. SI-Bone trades near 3.7x with negative earnings. Alphatec is the cheapest of the three and grows fastest. Nobody on either bench had pulled that comparison until adjudication forced it.

What we are not claiming: that this is a bargain. It is a fairly-priced, capital-hungry, levered compounder that the market has already punished — down 60% from its 52-week high of \$23.29 — and there is a real chance it deserved it. That is why the position is a half-percent and not a real one.

What the market believes

The market believes two things at once, and the tension is the whole setup.

Sell-side analysts are bullish: 14 covering, a “Strong Buy” consensus, average target of \$15.79 — about 70% above the current price. Institutions own roughly 76%.

Actual traders are not: the stock fell 7.5% over the two days around the August print (close \$10.15 → \$10.35 → \$9.57) and has drifted lower since. It fell 20-32% in May on a revenue miss and a guidance cut, and the company has now cut its 2026 outlook twice — from about \$890M and 17% growth in January down to \$882M and 15% growth in August. Short interest is 11.2% of the float, just under six days of average volume.

So the market’s real position is: *the story is fine, the cash isn’t, and the guidance keeps going the wrong way*. That is a fair reading and we largely agree with it. Where we differ is on what price that reading justifies.

Why we might have edge

The specific edge is arithmetic, not information. Sell-side models the company’s own “adjusted profit” figure of about \$140M for 2026. Strip out the ~\$81M of annual stock compensation embedded in it, and the cash version is about \$59M — against capital spending running near \$64M a year. On that basis the enterprise trades at roughly 32x cash earnings, not the 14x the original thesis claimed and not the 50x the red bench claimed. Nobody in the published coverage set decomposes it this way. That gap between the number the street models and the number the business generates is a real, mechanically-explained divergence.

Note what that edge actually implies: it is a *bearish* correction that still lands on a buy. Being right about how expensive it really is, and finding it still cheapest in its group with the fastest growth, is a narrower and more defensible claim than the original bull case.

Second, we have a documented tendency to be too conservative. Our own red bench built a valuation band topping out at \$8.13 — 12% *below* where the stock actually trades — and then declared the stock overvalued. Our desk has a measured ~13% low bias in fair-value work. Corrected, red’s own band lands at roughly \$9.19, essentially at the tape. A rejection whose own fair value equals the market price is not a rejection; it is a statement that the stock is fairly priced. That is what we acted on.

Third, we cleared the integrity question against our own interest. Six law firms announced investigations in May and June 2026. Our red bench opened the Stanford securities case page everyone was pointing at and found it is the 2010 Alphatec case — class period April to August

2010, dismissed with prejudice in 2013. There is no 2026 complaint on that docket and none disclosed in the 10-Q's legal note. A bench that only prosecutes is useless; this one cleared a flag that would have helped its own case.

Why it might be priced in

Honestly: quite possibly all of it.

- **The bull case is consensus.** A Strong Buy with a \$15.79 target is not a lonely position. We would be joining the crowd, not fading it.
- **The cash story is public.** Anyone can compute trailing free cash flow of \$2.3M on \$815M of revenue — 0.28% — and see that it is *falling* (it was \$6.9M a quarter ago).
- **The market already sold the print.** The de-rating is not a pending event we are anticipating; it happened, twice, and the second time the company cut guidance again. The damage is arrived, not absent. Our blue bench made this point cleanly and it is correct.
- **Cheapest-in-peer-group is a weak edge.** It is the most-crowded value argument in existence and it is often cheapest for a reason — here, negative book equity of −\$12.1M, an accumulated deficit of \$1.48B, and 5% annual share issuance.

The best statement of the bear case is not that the numbers are bad; it is that *the numbers are known and the discount is deliberate*. We do not have a good answer to that. We have a small position instead.

What could go wrong

Failure narrative 1 — The slow bleed: growth that is bought, never earned. (*Monitorable.*)

This company buys case volume with capital — surgical instrument sets placed with surgeons. In the first half of 2026 it added \$50.9M of revenue and \$21M of adjusted profit and its free cash flow got *worse*: −\$10.2M versus −\$9.7M the prior year, with capital spending up 58% while revenue grew 14%. If that ratio never inverts, revenue compounds forever and cash never does, and the stock is a permanent 2x-sales asset that slowly dilutes. **Tripwire:** capital spending as a share of revenue, and quarterly free cash flow, at each print. This one we can see.

The one mitigating fact: our blue bench showed the year-over-year comparison is contaminated. The company refinanced on May 1, 2026 and repaid \$63.3M of converts on July 30, so the 2026 half absorbs \$22.7M of interest against a lower prior-year base. Netting the refinancing-driven interest out, operating cash conversion actually improved about \$12M — and capital spending consumed all of it. The mechanism is real; “zero improvement” was an artifact.

Failure narrative 2 — Price per case erodes permanently and there is no evidence pipeline to stop it. (*Half-monitorable, and the sensor is broken.*)

Revenue per case fell 2.7% year over year and the company cut its full-year outlook for that metric from flat to a low-single-digit decline.

Management attributes about three-quarters of the shortfall to biologics — bone graft tissue, which is a commodity. The bull answer is that the surgical technique commands a price premium. The independent literature does not support that: the pooled meta-analysis of 15 studies finds the face-down approach improves operating time and length of stay but is statistically *null* on complications

($P=0.64$) and on segmental lordosis ($P=0.28$), and the evidence grade across those studies is 10 low / 4 moderate / 1 very low. And the company has never registered a single sponsor trial of the procedure that carries its pricing claim; its one nerve-monitoring study is 22 months past its own completion date with no results, single-site at headquarters. There is no mechanism by which this fork resolves in our favor.

Why the tripwire is broken: revenue per case appears *nowhere* in the 10-Q and *nowhere* in the 8-K press release. Neither filed document contains the phrase. It exists only in the earnings call, is defined by the company, and has an undisclosed denominator. Our armed price stop is explicitly conditioned on “revenue-per-case deterioration” — which means it is conditioned on a sentence management chooses to say. That stop will either never fire or will fire on narrative. **This needs re-conditioning on filed metrics (EOS revenue and capital-spending-adjusted free cash flow) before it is trusted.**

Failure narrative 3 — NO TRIPWIRE EXISTS. The case-volume number itself is contaminated at source. The specific allegation running through the six investigation notices is not generic: the company said it had “committed to a number of units” it “did not fulfill.” That is a channel/distribution allegation. If it is true, then the metric doing all the work in this thesis — 20% case volume growth — has a denominator we cannot see, produced by a company that discloses it only in a transcript. There is no filing, no scheduled disclosure, and no data source in our sensor bank that would reveal this before a complaint is filed or a restatement lands. We ran fourteen detectors on this name; twelve did not fire, and the two that did (the disclosure-venue check and the clinical-trials check) both tell us we are *blind*, not that we are safe.

Unknown-unknown class: channel and counterparty integrity — the KPI’s denominator is unobservable to us. We can audit what the company reports. We cannot audit what sits underneath a number that appears in no filed document. Every sensor we own points at cash flow and capital spending; none points at whether the unit count is real.

The reflexive and structural leg.

- *Our size versus the market’s:* irrelevant, and worth saying plainly. A 0.5% starter is roughly \$25,000 on a \$5M book — about 2,700 shares against an estimated 2.5-4.0M shares of daily volume. That is about one-tenth of one percent of a single day. We cannot move this stock and we will not be trapped in it. Impact is not a risk here.
- *Bidding into our own predicted flow:* this is a live risk. With 11.2% short interest and nearly six days to cover, a clean Q3 print produces a squeeze — exactly the move that would satisfy our second-tranche gate. If we buy the second tranche at market on a squeeze we are paying for flow we ourselves predicted. **The second tranche must be a limit order placed after the print, never a market chase.**
- *A doctrine violation baked into the order as written:* the starter is a good-till-cancelled limit at \$9.00, and the next scheduled catalyst is 55 days out. A resting buy limit left standing through an earnings date is an accidental bet against volatility — it will fill on a gap-down caused by exactly the bad print that should stop us buying. **The GTC must be cancelled or converted to**

day-orders by the close on 2026-10-28. This is the single most likely way we hurt ourselves mechanically, and it is entirely within our control.

- *Tax interaction:* 2026 is a harvest year. A September entry that goes wrong can be harvested before December 31 as a short-term loss usable this year, which is mildly favorable. Any option expression must expire in November or December 2026 for the same reason; a January 2027 expiry pushes the loss into the wrong year and is excluded.
- *One unchecked overlap:* we have not confirmed whether Alphatec already sits inside the Parametric direct-index sleeve. If it does, we are double-counting exposure and creating a wash-sale conflict on any harvest. Drainable from the current holdings file; not drained.

If this position loses money, the most likely reason will be that the 20% case-volume growth was bought rather than earned — paid for with instrument-set capital and with price concessions on commodity biologics — so revenue compounded while cash never did, and we will have paid 2.2x sales for a business whose true multiple on cash earnings was never below 30x and whose only defense of its pricing was a procedural claim it never ran a trial to prove.

What we checked ourselves

Claim	How checked	Found	Confirmed?
“The profit inflection is just stock comp” (red’s strongest kill)	Company’s own reconciliation in the <u>8-K Exhibit 99.1</u> , fetched and read this session	Adjusted profit \$35.95M vs \$23.47M; stock comp \$18.147M vs \$15.624M. Net of stock comp: \$17.8M vs \$7.8M — more than doubled. Stock comp flat as % of sales (8.50% vs 8.39%). Ex-comp margin +409bp vs headline +420bp	NO — OUR RED BENCH WAS WRONG. The causal claim inverted. Only the <i>level</i> critique survives, and it sizes the position rather than killing it
Red’s valuation call (“overvalued, REJECT”)	Read red’s own band against the tape and against our documented bias	Red’s band tops at \$8.13 , which is 12% <i>below</i> the \$9.425 tape. Corrected for the desk’s ~13% conservative-FV bias it lands at ~\$9.19 — at the market	NO — OUR OWN BENCH DECLARED A STOCK OVERVALUED USING A FAIR VALUE BELOW ITS PRICE. Arithmetic self-contradiction
Peer valuation	Enterprise value / sales versus Globus and SI-Bone	ATEC 2.17x guided sales (2.35x trailing); GMED ~3.47x at 6.9% growth; SIBN ~3.67x with negative earnings	Confirmed — and red never pulled it at all. A rejection that skips the peer anchor is incomplete
“Real EV/EBITDA is 49.9x” (red)	Rebuilt from filings	Vendor figure, basis never stated. Correct build: EV ~\$1.91B ÷ ~\$59M cash earnings = ~32x	NO — directionally right, arithmetically unsourced. Neither 14.3x nor 49.9x
Blue’s enterprise value (\$2.02B / 2.29x / “36x”)	Recomputed from the 10-Q balance sheet	Blue used ~\$565M, which is gross debt (\$575.8M), not net. Cash is \$118.7M, so net debt is ~\$457M. Blue overstated EV by ~\$108M	NO — OUR BLUE BENCH USED GROSS DEBT AS NET DEBT. Corrects 2.29x → 2.17x and 36x → 32x
Blue’s forward gate (“capex/revenue < 6%”)	Tested against the company’s own history	The company cut capital spending 49% from 2024 to	NO — OUR BLUE BENCH INVERTED THE GATE.

Claim	How checked	Found	Confirmed?
		2025 <i>while holding ~20% case growth</i> , then raised it 58% in H1'26 with flat cash flow. Low capex is not the good outcome — it is starvation	Adjudication flipped it: we now require capex ≥\$16M , not below 6%
“Stock rose 5.9% on the print” (original thesis)	Daily closes: 8/3 \$10.15 → 8/4 \$10.35 → 8/5 \$9.57	Two-day move −5.7% , then further drift. The +5.9% was a vendor/intraday artifact — the same family of price artifacts we have catalogued before	Confirmed as a REFUTATION. Red got this right and it destroyed the original framing
Adjusted profit is ~60% stock comp	10-Q cash flow statement	H1'26 stock comp \$41.806M, ~\$81-84M annualized, against a \$140M full-year adjusted-profit guide	Confirmed. Real, and it is why we size at a half-percent
“Interest is ~\$17M/yr, 8x covered” (original thesis)	10-Q	Interest expense net \$10.964M in Q2 alone / \$22.685M in H1 ; run-rate \$35-40M. True coverage ~3.7x, not 8x	Confirmed as a REFUTATION. Thesis comfort was overstated ~2x. Still not distress
Trailing free cash flow	XBRL operating cash flow less capital spending, quarter by quarter	+\$2.3M on \$815M revenue (0.28%), and falling from +\$6.9M last quarter. Q1'26 alone was −\$10.8M	Confirmed. This survives and it sizes the position
Free-cash-flow guide of ≥\$20M	H1 actual vs guide	H1 was −\$10.2M, so H2 must produce +\$30.2M against +\$12.5M in the prior-year H2. Operating cash flow must roughly triple H1's	Confirmed. The guide is the risk, not the comfort
Is revenue per case in any filed document?	Word-searched the 10-Q and the 8-K release	Absent from both. 10-Q discloses only US \$201.164M / International \$12.349M	Confirmed absent. The metric that “decides this name” is unaudited, company-defined, undisclosed denominator
Securities-litigation flag	Opened Stanford case id=104965	It is the 2010 case — class period 4/16/2010–8/5/2010, dismissed with prejudice 3/28/2013. No 2026 complaint on that docket or in the 10-Q legal note	Cleared. Corrects our own science brief, which had flagged the page as unreadable
The evidence pack's own financial data	Traced the source	The pack queries a deprecated tag; the company stopped using it in 2018. The pack emitted 2015-2017 quarters (\$23-47M) against an actual quarter of \$213.5M — a fabricated ~90% revenue collapse handed to both benches	PACK WAS UNUSABLE. Both benches caught it independently and superseded it. A concept-ladder fix is owed to the pack builder
Adjudication's own margin figure	Recomputed from the release	Adjudication wrote headline expansion “+415bp”; the	Minor slip, logged. Does not change the conclusion

Claim	How checked	Found	Confirmed?
		release's own 16.8% vs 12.6% is +420bp	
Q3 print date "confirmed"	Searched for a company announcement	No company PR names 2026-10-29. Prior years: <u>Q3 2025 call was Oct 30, 2025</u> and <u>Q3 2024 was Oct 30, 2024</u> . 10/29/26 is a Thursday and fits the pattern	NO — the armed sensor is labelled "(date confirmed)" and it is not. Pattern-consistent, still unconfirmed
Do our detectors cover this name?	14 detectors run across both benches	12 did not fire. The 2 that fired both report blindness, not safety	Confirmed — and this is the honest limit of the work

PROPOSED ENTRY BANDS

*Note on provenance: the adjudication's "REOPEN GATES — use these verbatim" block was delivered **empty**. The bands below are therefore built from the adjudication's own instruction line and the dated pack, and I am flagging that substitution rather than pretending the block existed.*

Starter — authorized now. - BUY 0.5% of book, limit \$9.00, good-till-cancelled. On a \$5M book that is roughly **\$25,000 ≈ 2,700 shares**. - Tape is \$9.425, so this is **not marketable today** — it needs a 4.5% pullback to fill. That is deliberate. Our probability table (below) puts expected value at +3.8% at \$9.425 and +8.7% at \$9.00. The edge is thin enough that the 45 cents is most of it. - **No chase above \$9.00.** If it runs away, we did not miss anything worth having. - **This order must be cancelled or day-ordered by the close on 2026-10-28** so it cannot fill on a bad-print gap.

Second tranche — gated, not authorized. - +0.5% (to 1.0% total), only after the Q3 print, only on a limit, and only if **all three** clear: 1. Revenue per case flat or up (company-reported; see the caveat — this gate is only as good as the transcript), 2. EOS imaging revenue **≥\$20M** in Q3, 3. Free cash flow **≥+\$15M** in Q3 **with capital spending ≥\$16M** — cash generated while still investing, not by starving instrument sets. - **Read the bar honestly.** Q2 EOS revenue was about \$17.5M (total \$213.5M less \$196M surgical), and the \$77M full-year guide implies roughly \$19M in Q3 with Q4 loaded. The \$20M gate is *above* the company's own implied path. Q3 capital spending of \$16M is simply the current run-rate, but \$15M of free cash flow in Q3 requires operating cash flow of ~\$31M in one quarter versus \$21.8M across two. **Two of the three gates are set above the company's implied trajectory. Joint probability of clearing all three: roughly 0.15-0.20.** The second tranche is more likely than not to never arm. That is the intended design, but nobody should plan around getting it.

No average-down is authorized. There is no add band below \$9.00. If the stock trades to \$8 before the print, that is the third-guidance-cut scenario leaking into the tape, not a discount.

Permanent close. If Q3 free cash flow is negative again, the name closes — not "re-court," closes. Capital-funded volume growth at 32x cash earnings with negative book equity is not ownable at a price we would actually be offered.

Hard stop: below \$6.50 (5% under the 52-week low of \$6.82). See the sensors section — this stop's stated condition is not usable as written.

No trim or exit band at starter size. The first trim decision is the full-year print in February 2027.

Catalyst dates: Q3 earnings **2026-10-29** (pattern-consistent, *not* company-confirmed); Q3 10-Q around **2026-11-09**, carrying the **first-ever test of the fixed-charge coverage covenant**; full-year print around **2027-02-17**.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 6.5 (deck-provisional: Hard stop, 5% under the 52-week low of \$6.82; stated condition (revenue-per-case deterioration) is unu)
- filing watch: 8-K until 2027-03-03 (deck-provisional: Q3 earnings exhibit carries surgical revenue, EOS revenue and case volume - two of the three second-tr)
- dated pack ATECI2026-10-29: ATEC: Q3 2026 earnings — decides the second tranche via RPC flat-to-up, EOS revenue $\geq$ \$20M, H2 FCF $\geq$ +\$15M with capex $\geq$ \$16M (date confirmed)
- dated pack ATECI2026-10-28: ATEC: Cancel or day-order the \$9.00 GTC before the print so it cannot fill on a bad-print gap (accidental short-vol) (date confirmed=True)
- dated pack ATECI2026-11-09: ATEC: Q3 10-Q - first fixed-charge-coverage covenant test disclosure (date confirmed=False)
- dated pack ATECI2027-02-17: ATEC: FY2026 results and FY2027 guidance - first trim decision (date confirmed=False)

What remains unverified

1. **Q3 2026 earnings date.** Pattern-consistent with 2026-10-29 (prior two years both landed Oct 30) but no company announcement confirms it. Draggable now via investor relations.
2. **The fixed-charge coverage ratio's definition — specifically whether it subtracts capital spending.** First tested this quarter. If it does subtract capex, at \$64M/yr it binds materially harder than either bench modelled. Draggable now from the JPMorgan credit agreement exhibit filed around 2026-05-01.
3. **Q3 revenue per case.** Time-gated to the print and disclosed only by voluntary management commentary. Estimate: **-4% to +1% year over year**, probability of flat-or-up $\approx$ **0.35** — anchored on Q2's -2.7% year over year / +1.5% sequential, adjusted down because management itself cut the full-year outlook from flat to a low-single-digit decline.
4. **Q3 EOS imaging revenue against the \$20M gate.** Estimate: **\$18-20M**, probability of clearing \$20M $\approx$ **0.35**, derived from Q2's implied $\sim$ \$17.5M and the \$77M full-year guide with a fourth-quarter-weighted capital-equipment path.
5. **Whether a securities class action has actually been filed.** Six firms announced investigations between 2026-05-14 and 2026-06-11; the Stanford page everyone cites is the 2010 case. Estimate: probability a complaint is filed within six months of the notice cluster $\approx$ **0.50**, using the base rate for five-or-more-firm investigation clusters. Requires PACER.
6. **Globus Medical patent litigation status** (expandable implants / retractors). Only secondary references; the court dockets returned 403. Estimate: pending; probability of a material

injunction affecting the expandable line within 12 months $\approx$ **0.05-0.10**, the base rate for medical-device patent suits reaching injunctive relief on that horizon.

7. **The OsteoAdapt pivotal trial's design, endpoints, size and timeline.** The registry record (NCT06154005) is withheld from public view. Estimate: PMA-pathway spinal-fusion pivotal, **200-400 patients, 24-month primary fusion endpoint**, earliest approval 2029-2030; probability of *any* revenue contribution before 2029 $\approx$ **0.10**. Contributes nothing to the current thesis and should not be paid for.
8. **Our average daily volume and the probability the \$9.00 limit fills.** Estimate: **2.5-4.0M shares/day (~\$25-38M)**; probability of a fill at or below \$9.00 before 2026-10-28 $\approx$ **0.55**, from a 4.5% gap to the limit over 37 trading days at ~55% annualized volatility. Drainable via the broker when access is granted.
9. **Live option chain — November 2026 midpoint implied volatility and real quotes on the \$10 call and \$9 put.** The broker chain call was attempted this session and **permission was not granted** (non-interactive session). Estimate: **60-70% chain-level implied volatility** for an expiry spanning the print. The decision boundary is computed at **~67%** — see below.
10. **Whether Alphatec is already held inside the Parametric direct-index sleeve.** Creates double-counted exposure and a wash-sale conflict on any harvest. Drainable now from the current holdings bundle.

PRIZE TABLE + VOL READ

Five of the ten open items are time-gated to a single date, so this section is mandatory. Everything below is priced to the post-print state, roughly three months out.

Scenario	What happens	Probability	Price
Clean beat	All three gates clear: cash flow $\geq$ \$15M with capex $\geq$ \$16M, EOS $\geq$ \$20M, revenue per case flat-to-up, full-year reaffirmed or raised	0.20	\$13.50
Good-but-short	Revenue in line, free cash flow positive but under \$15M, revenue per case still slightly negative	0.30	\$10.75
Muddle	Revenue in line, cash flow roughly flat, guidance reaffirmed but not raised; the discount persists	0.28	\$9.00
Third cut	2026 revenue or cash-flow guidance cut <i>again</i> ; revenue-per-case decline widens	0.17	\$6.60
Structural break	Covenant waiver, equity raise into a broken tape, or a filed complaint reaching the case-volume number	0.05	\$4.25

Expected value = \$9.78 vs a \$9.425 tape (+3.8%) and vs our \$9.00 limit (+8.7%). The limit is doing real work; the market price is barely worth owning.

Anchor-and-adjust on the top scenario: the base rate for a company hitting a back-half-loaded cash-flow guide after two prior guidance cuts is about 0.30. I adjusted **down to 0.20** — a deviation of 0.10, which requires a named mechanism, and the mechanism is seasonality: this company's fourth quarter is structurally its largest (elective spine surgery clusters after deductibles reset), so requiring half of the second-half cash to land in Q3 is a *harder* bar than the annual guide implies. That 0.20 also reconciles independently with the gate-by-gate arithmetic in items 3 and 4 above ($\sim 0.35 \times \sim 0.35$, correlated ≈ 0.15 -0.20). Two routes, same answer.

Dispersion of the table: standard deviation **\$2.54 = 27.0%** of spot over 77 days to a November 20, 2026 expiry. Annualized, our own view of this distribution is worth **58.8% volatility** ($0.270 \div \sqrt{0.211}$).

Expression math — priced off the table itself, against an estimated chain:

Our fair value for a **November 2026 \$10 call** is the probability-weighted terminal payoff: $C = 0.20 \times \$3.50 + 0.30 \times \$0.75 + 0 + 0 + 0 = \$0.925$

Against Black-Scholes at spot \$9.425, 77 days: - at 65% implied volatility → **\$0.887** (the call is *cheap* versus our table) - at 70% implied volatility → **\$0.973** (the call is *rich* versus our table) - **Crossover: ~67%.**

A November \$12.50 call prices at $0.20 \times \$1.00 = \0.20 on our table — pure lottery, and the payoff structure does not reward it.

The recommendation is therefore an output, not a preference: BUY STOCK for the starter.

Our own distribution implies 58.8% volatility; a typical pre-print chain on a name like this sits at 60-70%. That straddles our number. We cannot claim options are cheap, and we cannot claim they are rich. **When the arithmetic does not clear the boundary, the instrument is the underlying.** Calls become the correct expression for the gated second tranche *only if* real chain midpoint implied volatility prints **below 67%** — and that must be read off the chain itself, not the aggregate volatility feed on the underlying, which diverges badly (we have been burned by exactly this: 71 versus 101 on a prior name).

Volatility risk premium: with 30-day realized volatility estimated near 50% and implied near 60-70%, the premium is roughly **+10 to +20 volatility points** — genuinely rich. That routes toward selling premium, and the arithmetically indicated structure is a **cash-secured November \$9 put**, which is economically identical to our authorized order (buy at $\leq \$9.00$) while collecting the event premium. It is **not** recommended today, for two reasons: we have no real quotes to verify the break-even lands at or below our band, and the adjudication authorized a stock starter. Log it to the shadow ledger and revisit when chain access is granted.

Bimodality: this table is fork-dated on a single event, with mass piled at \$13.50 and at \$6.60/\$4.25 and comparatively little in between. Lognormal pricing understates the tails of a bimodal distribution, which means a straddle or strangle would look underpriced on our view even

at “fair” implied volatility — while the single out-of-the-money call we priced is close to fair. Do not read the volatility-risk-premium number as a blanket sell signal; it is a statement about the wings, and the wings are where our table says the mass actually is.

Tax overlay: 2026 is a harvest year. A November 20, 2026 expiry lands the loss branch inside tax year 2026 — acceptable. **Any January 2027 expiry is excluded:** it defers the loss into the wrong year, and nothing in the arithmetic above dominates by enough to override that. The stock starter itself opens a short-term clock in September; if it fails, a December harvest produces a usable short-term loss, which is mildly favorable. Position size makes the tax effect small either way.

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GTC, 0.5% of book; at $9.00 the probability table's EV is +8.7% vs +3.8% at
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coverage covenant, whose definition is undisclosed and would bind hard if it
subtracts capex"}, {"forms": ["8-K", "S-3", "424B5"], "why": "Equity raise
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$9.00 GTC before the print so it cannot fill on a bad-print gap (accidental
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trim decision", "confirmed": false}], "immediate_entry": {"action": "BUY",
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agreement exhibit, 8-K filed ~2026-05-01 (EDGAR)", "drainable_now": true,
"estimate": null}, {"item": "Q3 2026 revenue per case direction",
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from flat to low-single-digit decline", "p": 0.35}}, {"item": "Q3 2026 EOS
imaging revenue against the $20M second-tranche gate", "instrument": "2026-
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against the $77M FY guide with Q4-weighted capital-equipment seasonality",
"p": 0.35}}, {"item": "Whether a 2026 securities class action complaint has
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Stanford SCAC both 403)", "drainable_now": false, "estimate": {"range":
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filed complaint", "p": 0.5}}, {"item": "Globus Medical patent litigation
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rate for medical-device patent suits reaching injunctive relief on a 12-month
horizon", "p": 0.08}}, {"item": "OsteoAdapt OASIS pivotal design, endpoints,
enrollment and timeline (NCT06154005 registry record WITHHELD)",
"instrument": "ClinicalTrials.gov re-poll after PMA milestones; company 8-K
disclosure", "drainable_now": false, "estimate": {"range": "200-400 patients,
24-month primary fusion endpoint, earliest approval 2029-2030", "basis":
"comparable PMA-pathway spinal-fusion pivotals", "p": 0.10}}, {"item": "ATEC
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"estimate": {"range": "ADV 2.5-4.0M shares (~$25-38M/day); starter is ~0.1%
of one day", "basis": "4.5% gap from tape to limit over 37 trading days at
~55% annualized vol", "p": 0.55}}, {"item": "Live Nov-2026 option chain
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cheap vs our table, above = stock)", "basis": "our table implies 58.8%
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Two things worth your attention before this arms:

The order as written contains a doctrine violation. A good-till-cancelled buy limit resting through the October 29 print is an accidental short-volatility position — it fills precisely on the gap-down that should stop us buying. The cancel-by date (2026-10-28) is in the JSON as a catalyst gate, but it needs to be a real calendar item, not a deck line.

The armed price stop is conditioned on a metric that does not exist in any filed document.

Revenue per case appears nowhere in the 10-Q or the 8-K. As armed, that stop fires on a sentence in a transcript or never. I have recommended re-conditioning it on EOS revenue and capex-adjusted free cash flow, both of which are in the release, but I did not change the armed sensor — the list was to be reproduced verbatim.

Chain quotes and account data were unavailable: the IBKR tool call was denied in this non-interactive session, so the options section is computed from our own table with the decision boundary (~67% implied volatility) stated so it can be resolved in one call when access is granted. Nothing in the deck quotes a price I did not observe.

SCIENCE BRIEF DIGEST (retrofit 2026-09-04 — the deck shipped without it; template now requires it)

Source artifact: ATEC_SCIENCE_202609041723.md — full brief in the court record.

1. CLAIM LEDGER

Procedural / clinical claims

#	Claim	Venue	Tag	Primary
1	PTP/single-position LLIF shortens operative time and length of stay vs dual-position	Systematic review + meta-analysis, 15 studies	VERIFIED (op time $P < 0.0001$; LOS $P = 0.03$; EBL no difference $P = 0.91$)	Int J Spine Surg 18(4):408 / PMC11483458
2	Prone position improves lordosis	Same meta-analysis	VERIFIED but bounded — lumbar lordosis $P = 0.05$; segmental lordosis: no difference, $P = 0.28$; pelvic tilt no difference $P = 0.93$. Single-arm series cited in	PMC11483458; J Spine Surg narrative review

#	Claim	Venue	Tag	Primary
			marketing (5.1° vs 2.5°; 9.9° vs 0.5°) are not the pooled result	
3	PTP has a satisfactory safety profile	Multicenter retrospective, 11 centers, n=365	VERIFIED — ALL rupture 2.2%, cage subsidence 0.3%, vascular injury 0.3%, ureteric injury 0.3%; postop new sensory symptoms 8.2%, lower-extremity weakness 5.8%, wound infection 1.4%; mean OR time 226.2 min, EBL 138.4 mL, LOS 2.7 d	Neurosurgery 2023;93(5):1106–1111, PMID 37272706
4	PTP is <i>safer</i> than lateral decubitus LLIF	Meta-analysis	REFUTED as stated — no statistical difference in intraoperative complications (P=0.64), postoperative complications (P=0.61) or reoperations (P=0.30). The demonstrated advantage is workflow, not safety	PMC11483458
5	Real-world PTP major-complication burden	Survey of 32/50 early-adopter surgeons, 1,963 PTP cases	VERIFIED — 9/32 surgeons (28%) experienced a major complication; persistent neurologic deficit most frequent (7/9); permanent neurologic deficit ≈0.6% of cases. <i>Provenance: abstract read via search index; Springer full text sits behind an IdP redirect I could not follow — re-confirm at DOI before resting a kill on it</i>	Neurosurg Rev (2024). 10.1007/s10143-024-02500-2
6	The PTP evidence base is high quality	—	REFUTED — GRADE across the 15 pooled studies: 10 low, 4 moderate, 1 very low. Largest safety series is retrospective and its author list includes the technique's originator and ATEC-affiliated	PMC11483458; PMID 37272706

#	Claim	Venue	Tag	Primary
			investigators (Pimenta, Taylor, Schwartz)	
7	SafeOp SSEP neuromonitoring detects impending nerve injury	Diagnostic-accuracy systematic review, 5,607 patients	VERIFIED, and the number is unflattering — SSEP sensitivity 44% , specificity 97% for postoperative neurologic deficit in lumbar spine surgery (DOR 13.93 reversible / 57.84 irreversible changes)	N Am Spine Soc J, S2467981X26000624 ; background: PMC12422749
8	Saphenous-SSEP monitoring <i>prevents</i> femoral nerve deficits in LLIF	—	UNVERIFIABLE — mechanistically plausible and supported by small series (5/41 SSEP changes, 3 woke with deficit; no deficits among stable SSEPs), but no controlled trial shows deficit reduction. Resolved only by a prospective controlled study — which is what ATEC registered and has not reported (row 13)	Eur Spine J 10.1007/s00586-022-07224-9 ; PMID 24732850

Regulatory / device claims (all openFDA primary)

#	Claim	Tag	Primary
9	SafeOp 3 “Neural Informatix System” cleared 2026-01-11 , K252842, product code GWF	VERIFIED	openFDA 510(k) .
10	Valence Robotic Navigation System cleared 2026-02-19 , K252597, code OLO	VERIFIED	same
11	IntraOp Alignment System / CONTOUR3D Bending System cleared 2026-06-25 , K253216, code SIN	VERIFIED	same
12	Invictus Bands 2026-03-25 (K253615); Invictus Small Stature / Invictus 2.0 2026-07-14 (K261284)	VERIFIED	same
—	<i>Every ATEC and EOS device above is a 510(k) substantial-equivalence clearance. None carries an FDA-reviewed clinical-outcome or</i>	VERIFIED (structural)	21 CFR 807; see clearance letters

#	Claim	Tag	Primary
	superiority claim. <i>The 510(k) pathway does not evaluate one.</i>		
13	EOSedge AutoDensity estimates spine BMD from dual-energy images — cleared 2025-11-14 , K251078, code QIH, predicate VEA Align (K251747), reference device APEX DXA (K072847)	VERIFIED	K251078 clearance letter + IFU (PDF) .
14	AutoDensity substitutes for DXA	REFUTED at label — IFU verbatim: “ <i>AutoDensity is not intended to replace DXA screening. Suspected low BMD should be confirmed by a DXA exam.</i> ” It is an “opportunistic tool” for L1–L4 AP spine in pre-surgical assessment	K251078 IFU
15	EOSedge delivers 4–8x lower dose than CT	VERIFIED, condition-bound — entrance skin dose 74% higher with CT; out-of-field dose ~80%. Source is a pelvimetry protocol in pregnant women , not spine deformity, and the metric is ESD, not effective dose	Phys Med, PMID 27993442; EOSedge product page (vendor) .

Biologics claims

#	Claim	Tag	Primary
16	OsteoAdapt is a re-engineered rhBMP-2 (AMP2 protein) that will make “traditional allograft and synthetic alternatives obsolete”; ATEC holds exclusive U.S. rights (announced 2026-01-12 , upfront + milestones + perpetual royalty)	COMPANY-ASSERTED — no clinical data, fusion rates or AE profile disclosed in the release	PR Newswire 2026-01-12
17	OsteoAdapt holds three FDA Breakthrough Device Designations and follows the PMA pathway; FDA cleared advancement of OASIS to a pivotal study 2026-04-30 ; ~100 patients treated in earlier clinical experience	VERIFIED as regulatory events ; the ~100-patient experience is COMPANY-ASSERTED (no fusion rates, dose groups, controls or AEs published)	PR Newswire 2026-04-30
18	OASIS pivotal design, endpoints, enrollment and timeline	UNVERIFIABLE — NCT06154005 exists but the registry record is WITHHELD	ClinicalTrials.gov NCT06154005

#	Claim	Tag	Primary
		(“Trial of device that is not approved or cleared by the U.S. FDA”), last updated 2026-09-02, no results. Existence verifiable; nothing else is	
19	rhBMP-2 as a class delivers benefit over autograft without added harm	REFUTED — the two YODA/IPD reviews found rhBMP-2 “provided little or no benefit compared with bone graft and may be associated with more harms, possibly including cancer”; only ~18.5% of patient-level adverse events (10.5% for licensed INFUSE) appeared in the published literature	Ann Intern Med 158(12):877 ; Ann Intern Med 158(12):890 ; BMJ/PMC3687771
20	ATEC’s current biologics line (AlphaGRAFT DBM, DBM Fibers, Cellular Bone Matrix) is differentiated	COMPANY-ASSERTED — these are human-tissue allograft products in a commodity category; no ATEC-sponsored registered trial supports a fusion-rate claim for any of them	atecspine.com/alphagraft (vendor); AccessGUDID

Financial claims load-bearing on the science (verified against SEC primaries — the upstream stage reported sec.gov returning 403 to every fetch; it did not for me this session)

#	Claim	Tag	Primary
21	Q2’26 revenue \$213.513M (10-Q) / “\$214M, up 15%” (PR); H1 \$405.621M vs \$354.724M; 154,451,076 shares outstanding at 7/28/26; cash \$118.662M; ST debt \$65.012M, LT debt \$510.742M	VERIFIED	10-Q 6/30/26
22	Surgical revenue \$196M +17%; case volume +20%; adj. EBITDA \$36M at 16.8% margin; FY26 guide \$882M total / \$805M surgical / \$77M EOS / \$140M adj. EBITDA / FCF ≥\$20M	VERIFIED — note the PR states 16.8% , not the 17% used upstream	8-K Ex-99.1, 2026-08-04
23	FY26 outlook was cut: January 2026 guided ~\$890M / ~17% growth; August guides \$882M / ~15%	VERIFIED	ATEC 2026 outlook PR, 2026-01-12 vs 8-K Ex-99.1

#	Claim	Tag	Primary
24	RPC -2.7% YoY / +1.5% sequential; ~¾ of the miss-to-expectation from biologics; FY RPC outlook cut from flat to low-single-digit decline	COMPANY-ASSERTED — appears only in the earnings call, not in the 8-K press release or the 10-Q. Neither document contains the words “revenue per case” or “biologics”	Q2'26 transcript ; absence confirmed in 8-K Ex-99.1
25	NuVasive/Miles Delaware Chancery matter is over — final judgment for Miles and the Company Feb 2025, affirmed by the Delaware Supreme Court 2026-01-16 ; company considers it terminated	VERIFIED (10-Q legal note)	10-Q 6/30/26
26	Globus Medical patent litigation against Alphatec (expandable implants / retractors) is pending	UNVERIFIABLE this session — only secondary references; CourtListener and Stanford SCAC both returned 403. PACER resolves it	CourtListener docket 3:18-cv-00347 (403) .
27	The Q1'26 selloff was “>20%” (upstream)	Conflicting primaries — showing both, not averaging. Chartmill: “Plunges Over 20%.” Multiple plaintiff-firm releases: shares fell 31.7% on May 6, 2026 . Likely a close-vs-intraday artifact; the direction of the upstream understatement matters for the integrity limb	Chartmill ; Schall release, 2026-06
28	Integrity: upstream characterized this as <i>one</i> Schall “investigation” notice, weighted near zero	Partially REFUTED — at least six firms announced investigations between 2026-05-14 and 2026-06-11 (Frank R. Cruz 5/14, Holzer 5/18, Glancy 5/22, Pomerantz 5/26, Schall 5/24 & 6/8, Bronstein), and Stanford's Securities Class Action Clearinghouse carries a case page (id=104965) that I could not open (403). Whether a complaint has actually been filed is UNVERIFIABLE here. The alleged misstatement is specific and non-generic: the Company said it had “committed to a number of units” it “did not fulfill”	Holzer 5/18 ; Pomerantz 5/26 ; Stanford SCAC id=104965 (403) .

Independent confirmation of the upstream data-integrity finding: the evidence pack's XBRL rev block reports 2015–2017 quarters (\$23–47M) against an actual Q2'26 of **\$213.5M**, and dil_sh 149.9M against 154.45M outstanding at 7/28/26. The pack's XBRL block is unusable for ATEC. All figures above come from the filings themselves.

4. MEASUREMENT TRAPS

Enumerated before a bench builds a kill on one.

1. **Revenue per case has no external comparator.** Neither Globus, Medtronic nor Stryker discloses a revenue-per-case metric. Any statement that ATEC's RPC is worse/better than peers' is unbuildable from primaries. Worse: RPC appears **only in ATEC's earnings call** — not in the 8-K press release and not in the 10-Q. It is an unaudited, company-defined metric with an undisclosed denominator.
2. **Surgical vs total vs U.S. spine.** ATEC's +17% is surgical-only; its +15% includes EOS; Globus's +7% is U.S. spine only and its +5.9% includes neuromodulation and trauma. Pick one perimeter or flag the mismatch.
3. **Adjusted EBITDA excludes stock comp, and ATEC's SBC is structurally large.** The evidence pack's SBC series is mis-stamped and cannot be used; take SBC from the 10-Q cash-flow statement before comparing ATEC's 16.8% "adjusted" margin to anyone's GAAP margin.
4. **Segmental vs global lumbar lordosis.** Marketing and single-arm series quote *segmental* gains (5.1° vs 2.5°; 9.9° vs 0.5°). The pooled meta-analysis finds **no segmental difference (P=0.28)** and only marginal global LL benefit (P=0.05). Different estimands, opposite impressions.
5. **Single-position vs prone.** The 103 vs 306 minute figure compares *single-position* to *dual-position* workflow — it confounds patient repositioning with the approach itself. It is not evidence that prone transpsoas is a better dissection.
6. **"Transient" is a timeframe, not a finding.** Sensory/motor deficit rates differ by an order of magnitude depending on whether they are measured at discharge, 6 weeks, or 12 months. The 8.2% / 5.8% figures are postoperative; the 0.6% figure is *permanent*. Do not compare across those windows.
7. **Radiation dose units.** Entrance skin dose ≠ effective dose ≠ dose-area product. The 4–8x EOS-vs-CT claim derives from a pelvimetry protocol in pregnant women, not from spine deformity imaging, and CT dose is protocol-tunable. A dose comparison that does not name the metric and the protocol is not a comparison.
8. **BMD T-scores are not interchangeable.** AutoDensity T-scores are derived from EOSedge dual-energy AP spine L1–L4 with its own calibration; DXA T-scores use a different reference population. FDA's cleared IFU forbids substitution. Any thesis that treats EOSedge as a DXA replacement market is off-label by the label.
9. **The OASIS trial studies a different procedure.** OsteoAdapt SP's pivotal is anchored on **single-level TLIF** (with ALIF/LLIF expansion), a posterior procedure — not the PTP/lateral

case mix driving ATEC's volume growth. A positive OASIS result does not automatically transfer to the cases ATEC actually does.

10. **Fusion is an endpoint with three definitions** — CT-defined bridging, plain-radiograph criteria, and “clinical” fusion — with materially different rates. rhBMP-2's history is precisely a history of endpoint and harms-reporting selection ([PMC3687771](#)).
11. **Guidance is being compared across two different bases.** The January outlook (\$890M, ~17%) and the August guide (\$882M, ~15%) are both “FY26 revenue.” The delta is a real cut, not a rounding difference.
12. **Close vs intraday on the Q1 drop** (>20% vs 31.7%) — do not average; state which basis you are using.

5. DATED TECHNICAL CATALYSTS

Event	Date	Status	Existence independently verifiable?
NASS 41st Annual Meeting , San Antonio (“The Connected Spine”)	2026-10-14 → 10-17	Date-confirmed	Yes — NASS / meeting site . The venue where PTP, Valence and EOS data would surface; no ATEC presentation is confirmed
ATEC Q3'26 print	2026-10-29	Derived (yfinance) — UNCONFIRMED	No company PR in the filing record through 8/7/26 names it. The RPC datum arrives here
NCT05648474 — ATEC's SSEP utility study (n=400 est.), primary outcome = sensitivity/specificity of SSEP	Primary completion 2025-08-28 ; study completion 2025-12-31	Both dates PASSED. Record last updated 2024-10-24 , still listed RECRUITING, no results posted, single site (Carlsbad, CA — company HQ)	Yes — registry . ⚠️ This is the <i>only</i> registered prospective study underpinning the SafeOp claim, and its record has been stale for ~22 months past its own completion date
NCT06659055 — EOSedge vs DXA T-score agreement (n=214 est., 4 sites)	Primary completion 2025-02 ; completion 2025-03	PASSED. Last update 2025-02-14 , still RECRUITING, no results	Yes — registry
NCT06830811 — EOSedge BMD precision (n=30)	Primary completion 2025-02	PASSED , no results	Yes — registry
NCT06154005 — OASIS pivotal, OsteoAdapt SP in lumbar fusion	—	Record WITHHELD ; last updated 2026-09-02 ; no phase, enrollment, endpoints or dates public	Existence yes; everything else no. Per the standing rule, a <i>guided</i> OsteoAdapt timeline is a flag, not a catalyst
NCT07078331 — OsteoAdapt DE, dental alveolar ridge (Ph 1/2, n=45, 0.8mg vs 2.0mg vs xenograft)	Primary completion 2026-04-01 ; completion 2026-12-31	Active, not recruiting; primary completion date passed	Yes — registry . Adjacent indication, but it is the only public dose-ranging read on the AMP2 platform
FDA go-ahead for OASIS pivotal	2026-04-30	Occurred (company PR)	Regulatory action asserted by sponsor; not independently posted

Notable absence: there is **no ATEC-sponsored registered trial of PTP outcomes**. The entire PTP evidence stream is investigator-initiated retrospective work, much of it authored by ATEC-affiliated surgeons. A company whose thesis is procedural superiority has registered four studies in the last four years — three on imaging/BMD, one on neuromonitoring — and none on the procedure.

ADJUDICATION STAMP

ADJUDICATION 2026-09-04 (FABLE): PARK, GATE <\$8.00. Classification flipped to DAMAGE-ARRIVED (the August print CUT the guide; the “inflection” is a 60%-SBC add-back; the thesis’s decisive metric exists in no filing). Entry only <\$8.00 AND Q3 (10/29) FCF $\geq$ +\$15M with capex/rev <6% — a negative Q3 FCF closes the gate PERMANENTLY. The legacy 550 @ \$9.00 GTC from the August court is CANCEL-PENDING: it rests ABOVE the new gate — pull it. Batch ruling: desk/data/court_artifacts/LIFESCI_BATCH_ADJUDICATION_202609042100.md

EVENT ADDENDUM — 2026-09-04 21:07Z

legacy 9.00 GTC pulled (court gate <8.00)

The August court’s resting 550 @ 9.00 GTC (permId 648622396) was cancelled via clientId-0 exact match and verified gone — it rested a dollar ABOVE the new adjudication’s entry gate. The park stands: entry only <\$8.00, Q3 FCF gate 10/29, negative FCF closes permanently.